

Standing Up a PMIS in Healthcare IT

By Sid Bhardwaj

Who this is for: newly appointed healthcare IT project managers and PMO leads who have just been asked to formalize project management at their organization for the first time, and who need a practical, evidence-based way to decide how to do it, not a textbook.

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This guide walks newly appointed healthcare IT project managers and PMO leads through the process of standing up a formal Project Management Information System (PMIS) for the first time. If you are reading this, there is a good chance you were just handed responsibility for centralizing how your organization manages IT projects, probably after something like a delayed EHR module or an integration that went over budget forced leadership to ask for more visibility. You already know how to run a project. What you likely have not done yet is choose or configure a formal PMIS, build the case for a PMO to hospital executives, or account for the healthcare-specific factors, like regulatory compliance, clinical buy-in, and maturity models such as EMRAM, that change what good project management looks like in a hospital compared to general corporate IT. This guide follows the order you will actually move through as you make that decision.

1. Getting Oriented: What a PMIS Actually Is

Before making any decisions, it helps to get the terminology straight, since PMIS is a term people throw around loosely. At its core, a PMIS is the combination of tools, processes, and data used to plan, execute, and report on projects, not just a piece of software you buy. [Wikipedia's entry on the topic](#) draws a useful distinction here, separating a PMIS from a general project management system: a PMIS specifically centers on the information and reporting side, things like status data, resource allocation, and documentation, rather than just task execution. [TechRepublic's guide](#) builds on this by walking through how a PMIS functions across the entire project lifecycle, from initiation through closeout, covering practices like setting clear objectives before you ever configure a tool. Together, these two sources establish the shared vocabulary needed before evaluating any specific PMIS option.

2. Building the Case for a Formal PMO

Once you understand what a PMIS is, the next step is usually pitching formal project governance to people who have gotten by without it. This is where healthcare specifically diverges from general corporate IT. A [PMI.org paper on healthcare IT PMOs](#) explains that governance models, staffing patterns, and reporting lines in a hospital PMO look structurally different from a PMO in, say, a retail or finance company, largely because of how clinical stakeholders and compliance requirements insert themselves into every decision. It helps to pair that conceptual explanation with something concrete. The [University of Iowa Health Care PMO](#) publishes its own standards, status-reporting templates, and consultation services publicly, giving readers an actual, currently operating example to point to instead of an abstract description. When building a business case, showing executives a real academic medical center doing this successfully lands better than theory alone.

3. What the Evidence Actually Says

Before choosing a tool or process, it is worth grounding decisions in actual evidence rather than vendor claims or internal opinion. A [PMI Global Congress survey of 100 project managers](#) found that in practice, users value certain core PMIS features far more than others, features that do not always match what marketing materials highlight, revealing a persistent gap between what vendors promise and what practitioners say actually helps them. A [more recent 2024 paper](#) looks specifically at how PMIS platforms support real-time cost and

technical oversight, essentially how dashboards let managers catch budget and schedule risk before it becomes a crisis rather than after. Reading these together, the pattern that emerges is that reporting and oversight functionality tends to matter more to practitioners day to day than flashy features, which should shape how readers weigh their options in the next section.

4. Choosing Your Tool: Build, Buy, or Adapt

This is usually the point where people jump straight to comparing vendor feature lists, but it helps to first understand the real trade-off between building on an open-source tool versus buying a commercial platform. [OpenProject](#) is a fully open-source PMIS, and because its entire codebase and architecture documentation are public on GitHub, a team can actually see how its data model, permissions, and reporting modules work before committing to it, something not possible with most closed commercial products. On the other end, a [Planview case study on Parkview Health](#), a nonprofit health system, shows what adopting a commercial, vendor-supported platform looks like in practice: they used it to replace an informal process where almost every project request got approved regardless of actual business justification or available capacity. Neither approach is universally right. The decision comes down to an organization's technical capacity and its appetite for vendor support versus internal control.

5. Reporting Progress the Way Executives Expect

Once a PMIS is running, project managers need to report progress in language executives already understand, and this is where the [PMI EHR Deployment Best/Worst Practices paper](#) and HIMSS's [EMRAM model](#) (the Electronic Medical Record Adoption Model, a widely used benchmark for how far a hospital's EHR capabilities have progressed) do different but complementary jobs. The PMI paper draws on post-mortems of real EHR deployments to show what separated the rollouts that succeeded from the ones that did not, and one recurring theme is that successful projects reported progress against milestones tied to a defined maturity path rather than against an internal project plan alone. That is exactly the gap EMRAM fills: it gives hospital boards a maturity benchmark most executives are already somewhat familiar with, so mapping project milestones to EMRAM stages translates technical progress into something leadership can act on.

Here is what that mapping actually looks like in practice. If your project includes rolling out computerized provider order entry (CPOE) to every clinician licensed to place orders, alongside a second tier of evidence-based clinical decision support, you are not just closing a task on a Gantt chart, you are completing the core requirement of EMRAM Stage 4. If the next phase adds bar-coded, closed-loop medication administration, tying an electronic order to an identified nurse, a scanned medication, and a scanned patient, that milestone maps to Stage 5. Framing status updates this way turns a report that says "CPOE go-live is 80% complete" into one that says "we are on track to close out Stage 4 by Q3, which unlocks the clinical decision support capability the board asked about last quarter." That second version is the one that gets budget approved.

6. Learning From People Already Doing This

Everything up to this point has been process and evidence. It helps to end with what it actually looks like when a healthcare PMO gets this right, because a maturity model and a best-practices paper can tell you what to do without ever showing you what doing it well feels like day to day.

In a [Projectified podcast episode](#), the director of the enterprise project management team at Moffitt Cancer Center, winner of PMI's 2021 PMO of the Year award, describes how her PMO stayed focused on patient impact even while reorganizing priorities mid-pandemic. What comes through in that conversation is not a framework, it is a habit: every project decision got run back through the question of what it meant for patients, which is a useful gut check for any PMO leader building intake criteria, since it is easy for a new PMO to default to ranking projects by cost or executive sponsor rather than by patient impact.

Zoom out from one PMO's story to the industry level and [Becker's Hospital Review's year-end roundup](#) of major health system IT projects shows the same pattern repeating at scale: Adventist Health's Epic rollout across 27 hospitals, Northwell Health's enterprise EHR implementation, and similar projects at other large systems all involved the same ingredients this guide has walked through, formal governance, tool decisions made deliberately rather than by default, and progress reported in terms executives could act on. The scale is different from what a single newly formed PMO will manage in its first year, but the underlying discipline is the same one this guide has been building toward since Section 1.

If you take one thing from this guide, take this: a PMIS is not the goal. It is the infrastructure that lets a PMO consistently make the kind of patient-focused, evidence-based decisions Moffitt's team describes, at whatever scale your organization operates.

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If you only have twenty minutes today, do these five things in order. First, skim the Wikipedia and TechRepublic pieces from Section 1 so you have a two-sentence definition of PMIS ready for your next conversation with leadership. Second, pull up the University of Iowa PMO page from Section 2 and note two or three of their status-reporting templates as talking points for your own pitch. Third, look at which single PMIS feature the PMI survey in Section 3 found practitioners value most, since that becomes your first evaluation criterion once you start comparing tools. Fourth, decide, even provisionally, whether your organization leans toward OpenProject's self-hosted control or a vendor-supported model like the one Parkview Health adopted in Section 4. Fifth, find out which EMRAM stage your organization is currently sitting at, since every milestone you report from here forward should be framed against it. None of these five things require a finished PMIS. They just require you to stop reading and start asking questions inside your own organization.

Sources

Cited inline above; this guide draws on twelve sources from a larger [18-source curated collection on PMIS in healthcare IT](#), which covers additional ground this guide didn't need for a first-time getting-started audience.

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